

Communication Skills – Calgary Cambridge Guideline of Medical Interview – Gathering Information

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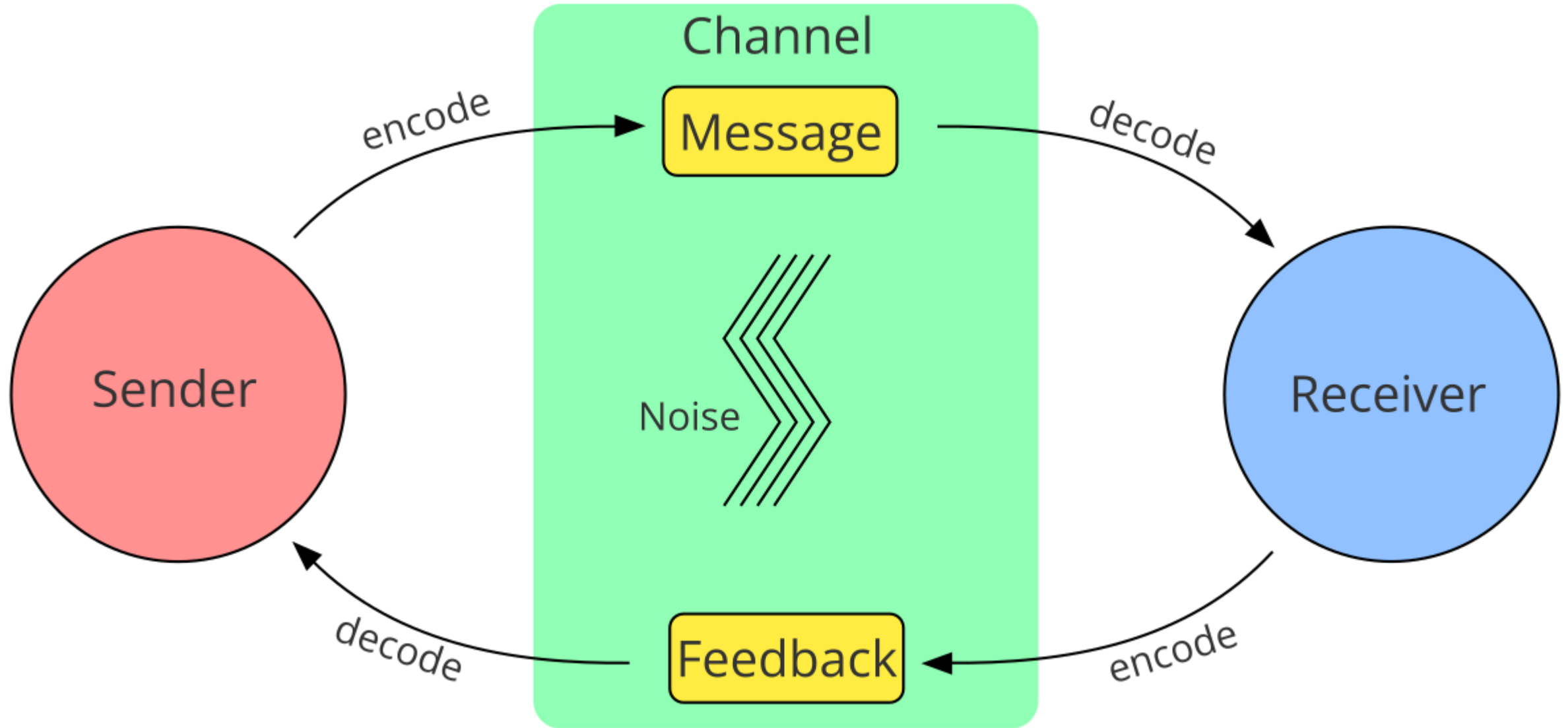
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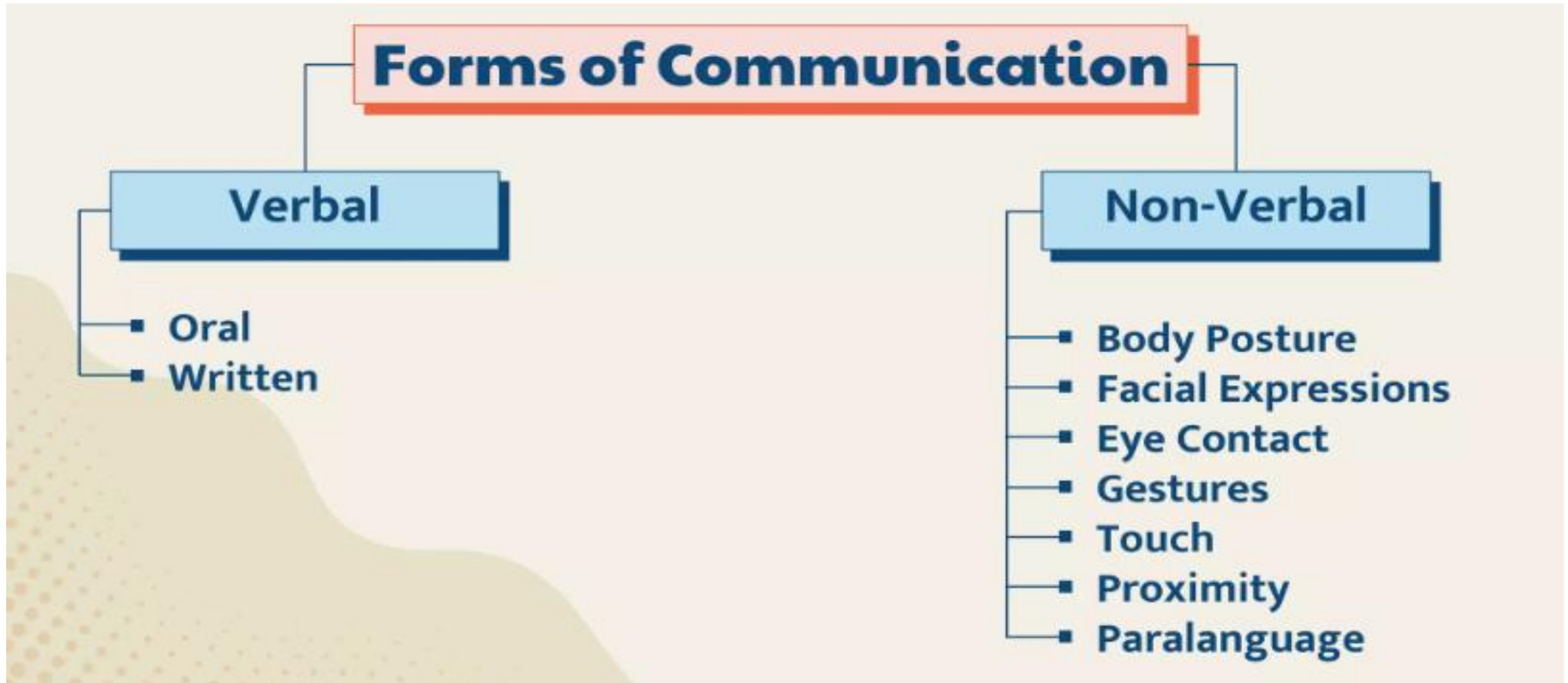
Lecture objectives

- **Understand the psychosocial aspects of patient**
- **Analyze disease-lines model**
- **Describe the steps of effective gathering information**

COMMUNICATION

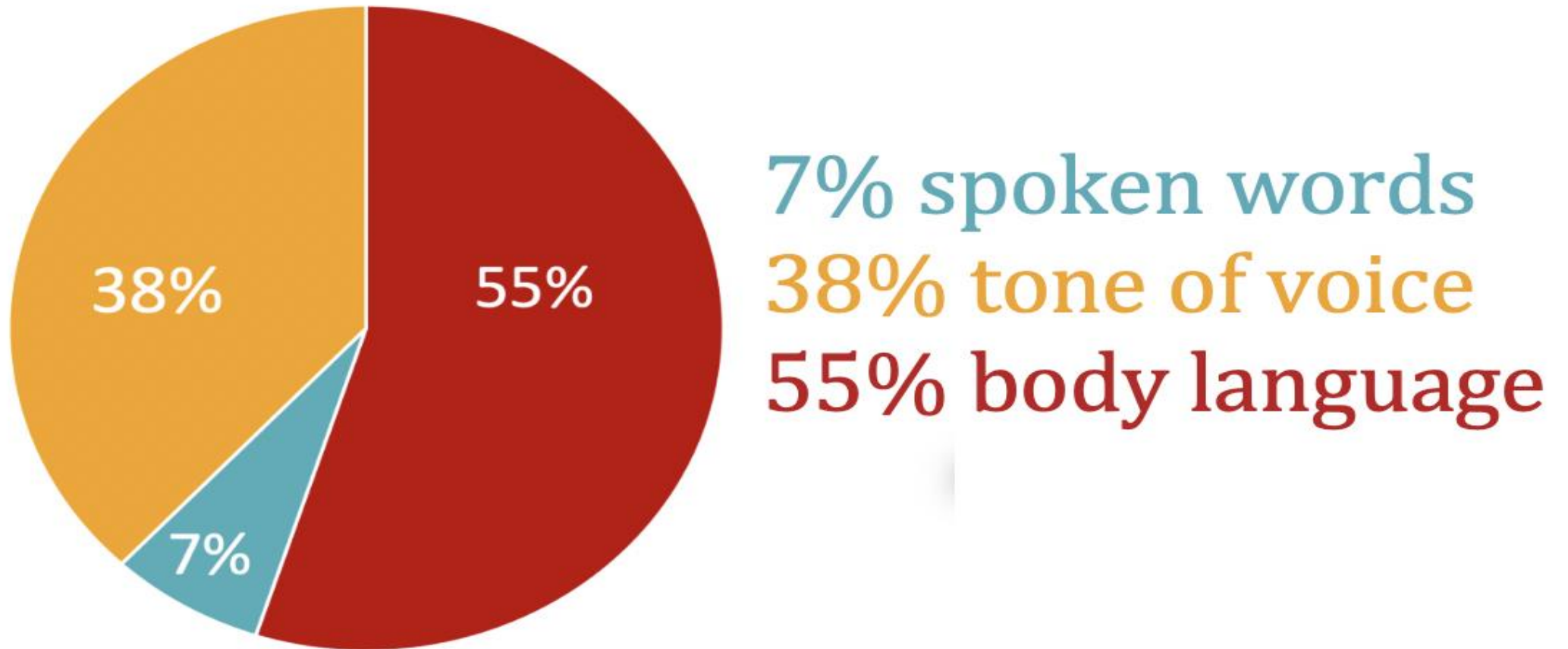


COMMUNICATION



COMMUNICATION

Dr Albert Mehrabian's 7-38-55% Communication Rule
Elements of Personal Communication



DOCTOR-PATIENT COMMUNICATION

Despite advancements, Effective communication remains physicians single most efficient tool!

DOCTOR-PATIENT COMMUNICATION

Despite advancements, **Effective communication remains physicians single most efficient tool!**

BENEFITS

- Improves health outcome
 - Enhances treatment adherence
 - Greater patient satisfaction
 - Better understanding and comprehension
 - Reduces anxiety and stress
 - More personalized care
- More accurate diagnoses
 - Better treatment plans
 - Increased job satisfaction
 - Improved efficiency
 - Reduced malpractice risk

BARRIERS

- Physiological barriers
- Psychological barriers
- Environmental barriers
- Time barriers
- Language barriers
- Cultural barriers

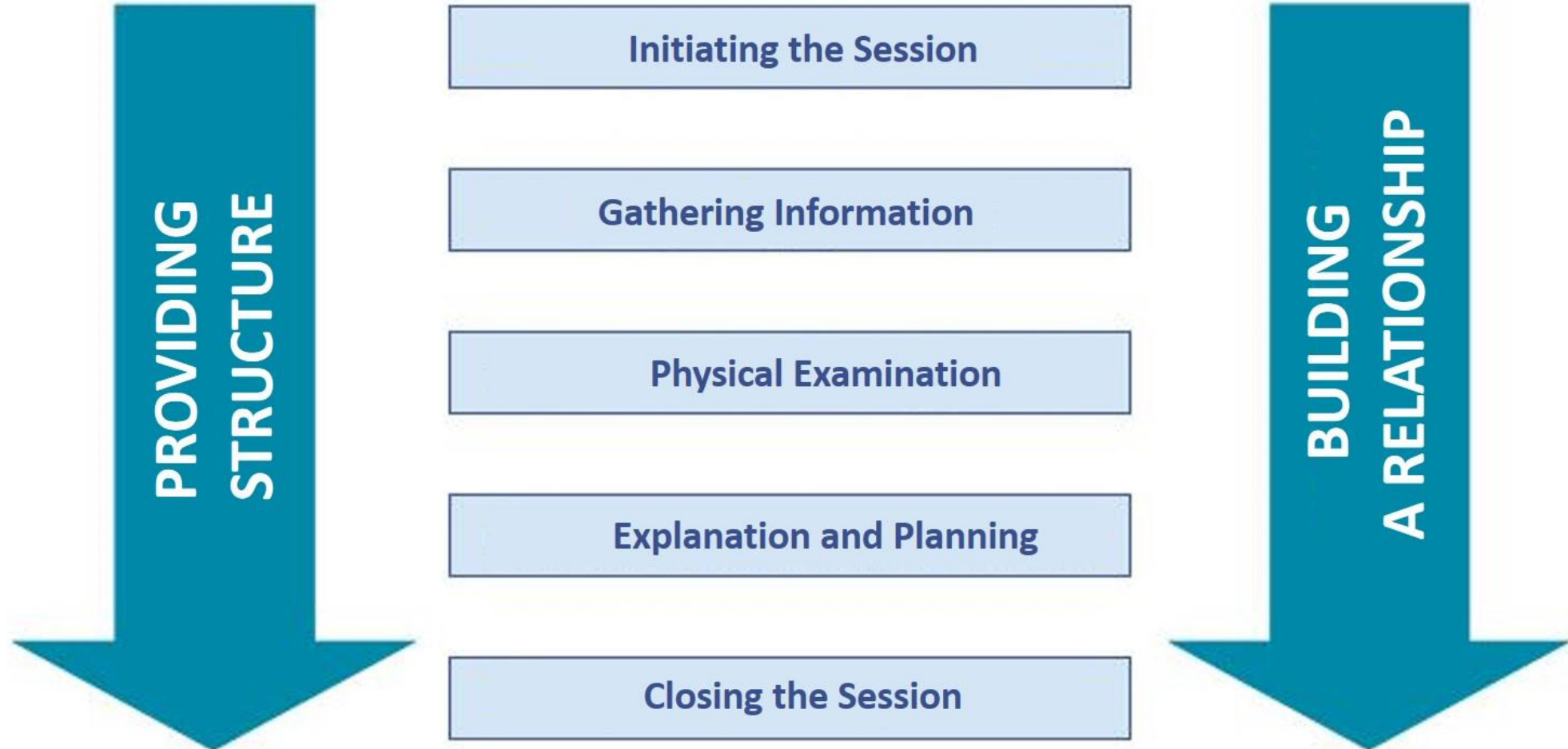
IMPROVEMENTS

- Active listening
- Appropriate body language
- Using clear speech
- Confrontation
- Questioning
- Empowering the patient
- Eliciting patient's perspective
- Empathy

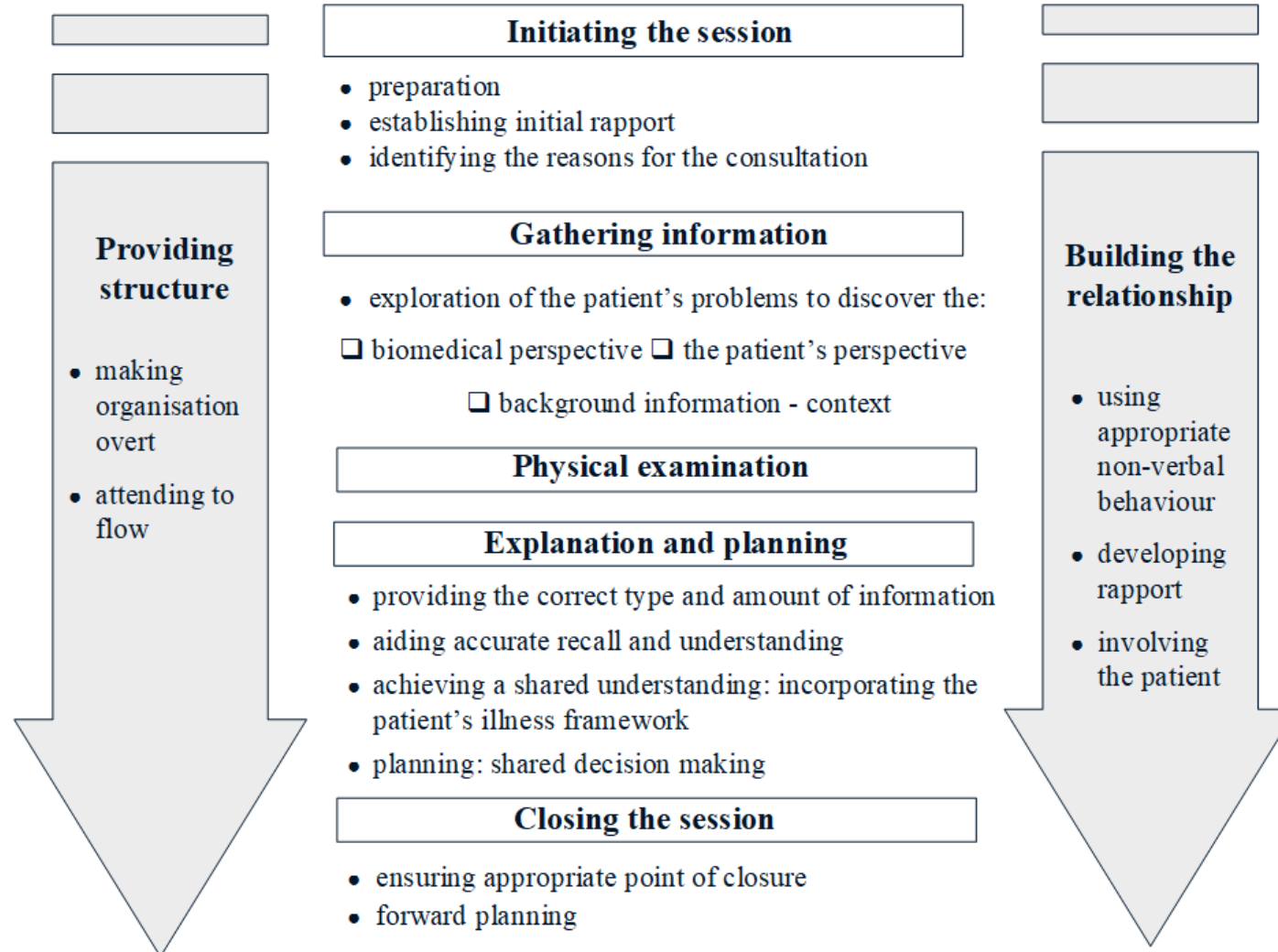
How to deal with patient problem?

- Data input = Gathering information + Physical Examination
- Data processing = Providing Structure (Patient agenda + Doctor agenda)
- Data output = Explanation and planning

Calgary Cambridge guideline of medical interview



Calgary Cambridge guideline of medical interview



Calgary Cambridge guideline of medical interview – Gathering Information

How can we successfully gather the information?

- Follow the disease illness model
- The scientific background + Patients psychosocial manner

Disease – Illness model

**Disease
(Biomedical
Perspective)**

Illness (Patient Perspective)

**Ideas
Concerns
Expectations
Believes
Effects on Life**

Disease – Illness model

**Decide to eliminate
the pathology only
(Treating the
disease)**

**Decide to restore the
wellbeing in the context of
patient's life (Treating the
patient)**

Disease – Illness model

**Passive recipient of
treatment**

**Active involvement in the
treatment**

Disease (The Biomedical Perspective)

- **What it is:** The **objective, pathological process in the body's structure or function**. It is what doctors are trained to diagnose and treat.
- **Focus:** Biological malfunctions, pathogens (viruses, bacteria), biochemical imbalances, genetic abnormalities, and anatomical defects.
- **Evidence: Measurable** by tests like blood work, biopsies, X-rays, MRI scans, and physical exams.
- **Example:** A physician diagnosing "Type 2 Diabetes" based on a hemoglobin A1c level of 7.5%, elevated fasting glucose, and symptoms of polyuria.

Illness (The Patient's Perspective)

- **What it is:** The **subjective, personal experience of being unwell**. It encompasses how the patient feels, lives with, and responds to the disease.
- **Focus:** Symptoms (pain, fatigue, nausea), emotional responses (fear, anxiety, depression), and the impact on daily life, relationships, and social roles.
- **Evidence:** Reported through the patient's story, their concerns, and their behavior.
- **Example:** The same patient with diabetes experiences the illness as constant thirst, fatigue that prevents them from playing with their grandchildren, anxiety about giving themselves injections, and financial stress from the cost of medication.

The Psychosocial Perspective

- The psychosocial perspective **is a framework that expands the view of health beyond just biology**. It argues that to fully understand health and disease, we must consider the intricate interplay of:
 - Psychological Factors: The mind, emotions, and behavior.
 - Social Factors: Cultural, socioeconomic, and environmental influences.
- This perspective is essentially the "engine" that connects the disease and illness models, explaining why and how the two often differ.

Biopsychosocial theory

Physiological perspective



```
graph TD; A[Physiological perspective] --- B[Psychological components]; A --- C[Social components]
```

The diagram illustrates the three components of Biopsychosocial theory. At the top is a blue rounded rectangle labeled 'Physiological perspective'. Below it, to the left, is another blue rounded rectangle labeled 'Psychological components', and to the right is a third blue rounded rectangle labeled 'Social components'. All three components are connected by a single horizontal line, indicating their interrelated nature in the theory.

Psychological components

Social components

Psychological Components:

- This refers to the individual's internal world:
- **Cognition:** Beliefs about the disease (e.g., "Cancer is a death sentence" vs. "This is a challenge I can fight").
- **Emotions:** Stress, anxiety, depression, fear, anger, and hope.
- **Behavior:** Health-related behaviors (adherence to medication, diet, exercise, smoking) and coping strategies (seeking support vs. avoidance).
- **Personality:** Traits like optimism/pessimism or resilience can influence both the risk of disease and the experience of illness.

Social Components:

- This refers to the individual's external world and relationships:
- **Socioeconomic Status:** Income, education, and occupation. Lower SES is strongly linked to higher disease risk due to factors like stress, limited healthcare access, and poorer living conditions.
- **Culture:** Cultural beliefs shape how symptoms are perceived, what kind of help is sought (e.g., traditional healer vs. medical doctor), and how patients communicate with providers.
- **Social Support:** The presence or absence of a network of family, friends, and community. Strong support is linked to better recovery and coping.
- **Stigma:** Societal attitudes toward certain diseases (e.g., mental illness, HIV/AIDS) can create shame and isolation, worsening the illness experience.
- **Environment:** Living in a food desert (lack of fresh food), or in an area with high pollution or violence, contributes to disease risk.

How It All Fits Together: A Practical Example

- Let's use **Coronary Heart Disease (CHD)**.
- The Disease (Biomedical): Atherosclerosis—the buildup of plaque in the coronary arteries, measurable by an angiogram.
- Treated with stents, statins, and beta-blockers.

How It All Fits Together: A Practical Example

- Let's use **Coronary Heart Disease (CHD)**.
- The Illness (Patient Experience): Chest pain (angina), shortness of breath, fear of having a heart attack, inability to climb stairs, anxiety about daily activity.

How It All Fits Together: A Practical Example

- Let's use **Coronary Heart Disease (CHD)**.
- The Psychosocial Perspective (The Connecting Lens):
 - Psychological: A type-A, high-stress personality may have contributed to the disease's development. Post-diagnosis, depression is common and can lead to poor medication adherence. The patient's belief that "I'm now broken" can hinder rehabilitation.

How It All Fits Together: A Practical Example

- Social: A low-income patient might live in a neighborhood without safe parks for walking (exercise), struggle to afford healthy food and expensive medications, and have a job that doesn't offer sick leave for doctor's appointments. This social context directly fuels the disease process and worsens the illness experience.

How to explore patient's perspective (illness)?

- Ask about?
 - **Ideas**
 - **Concerns**
 - **Expectations**
 - **Believes**
 - **Effects on life**
-
- + The list of problems, exploring the symptoms, past history etc...

How to explore patient's perspective (illness)?

What happened? (Patient narrative)

Why did it happen? (Their beliefs about the possible causes)

Why now? (Why the symptoms started and they did)

Why me? (Personal and/or cultural significance)

What if nothing is done? (Fears about the outcomes)

What should be done? (Expected or desired interventions)

- + The list of problems, exploring the symptoms, past history etc...

Conclusion

- **Balance the biomedical and illness perspectives: both are essential**
- **Time management: while it takes time, but it prevents misdiagnosis and non-adherence**
- **Continuous skill development**

- Thank You